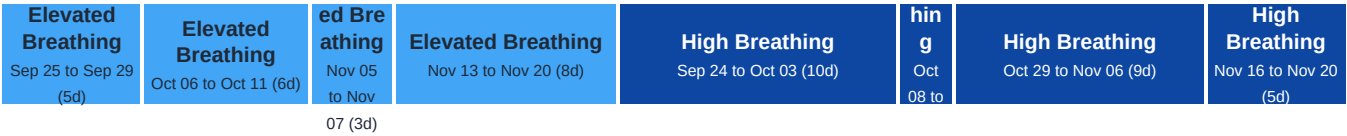


Patient ID: RSanchezPeriod: September 17 to November 20, 2024Report Date: November 21, 2024Coverage: Bed: 1163/1544h (75.3%)Chair: 1117/1544h (72.3%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 65 days from Sep 17 to Nov 20

■ HR ■ Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	57 bpm	49 bpm	71 bpm
Breathing	19 brpm	9 brpm	56 brpm

Limited data in the final 24h (15/24h); values reflect available readings.

Last 24 hours: two high breathing episodes in the afternoon; two elevated breathing episodes in the afternoon.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Nov 20	4 PM to 5 PM	4h	4	Elevated Breathing	26	17	45	Check SpO2 and lung sounds; assess for fluid overload or early infection
Nov 20	2 PM to 3 PM	2h	2	High Breathing (brief)	30	14	56	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

86 episodic events Detected, spanning 4 days total. Conditions: Elevated Breathing, Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Breathing. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
86	100h	High Breathing	1	100%

Major Findings: Sustained high breathing (avg 35, peak 58)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Sep 24	1 PM to 2 PM	15h	12	High Breathing	35	12	58	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Oct 08	8 AM to 9 AM	3h	3	High Breathing	35	12	54	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Oct 29	9 AM to 10 AM	20h	16	High Breathing	35	16	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Nov 16	12 AM to 1 AM	7h	7	High Breathing	34	14	56	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Sep 25	7 AM to 8 AM	6h	6	Elevated Breathing	26	14	57	Check SpO2 and lung sounds; assess for fluid overload or early infection
Oct 06	1 PM to 2 PM	12h	10	Elevated Breathing	26	13	46	Check SpO2 and lung sounds; assess for fluid overload or early infection

+ 2 additional condition(s) across Elevated Breathing; details in monitoring data.

The P5 to P95 heart rate spread was 24 bpm (52 to 76), indicating significant cardiac variability.

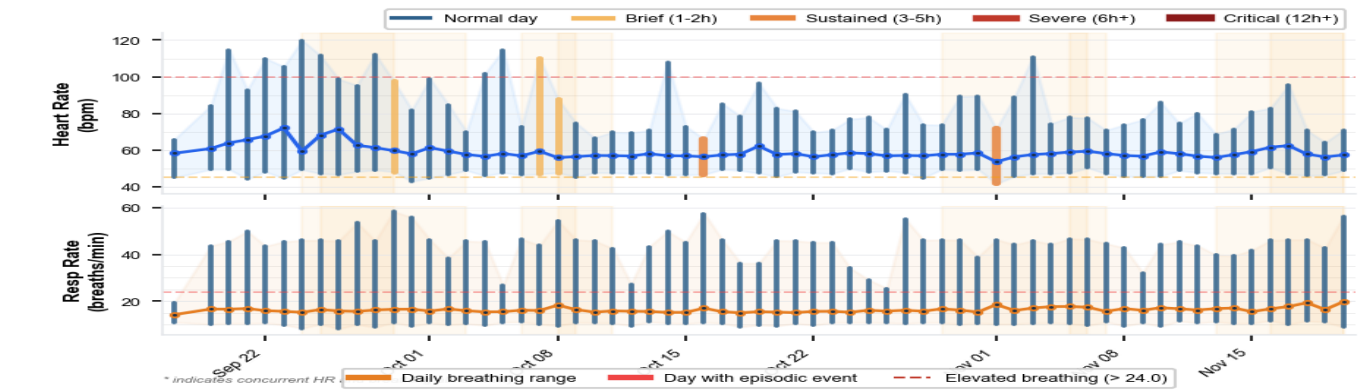
Suggested Clinical Review Actions

- High Breathing (avg 35 brpm, peak 46 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Elevated Breathing (avg 26 brpm, peak 46 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 103 bpm, peak 115 bpm): Check temp, hydration, pain; review for arrhythmia or infection

- Elevated Heart Rate (avg 98 bpm, peak 120 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Very High Breathing (avg 44 brpm, peak 54 brpm): Check SpO₂, work of breathing, fever; consider pulmonary cause or CHF

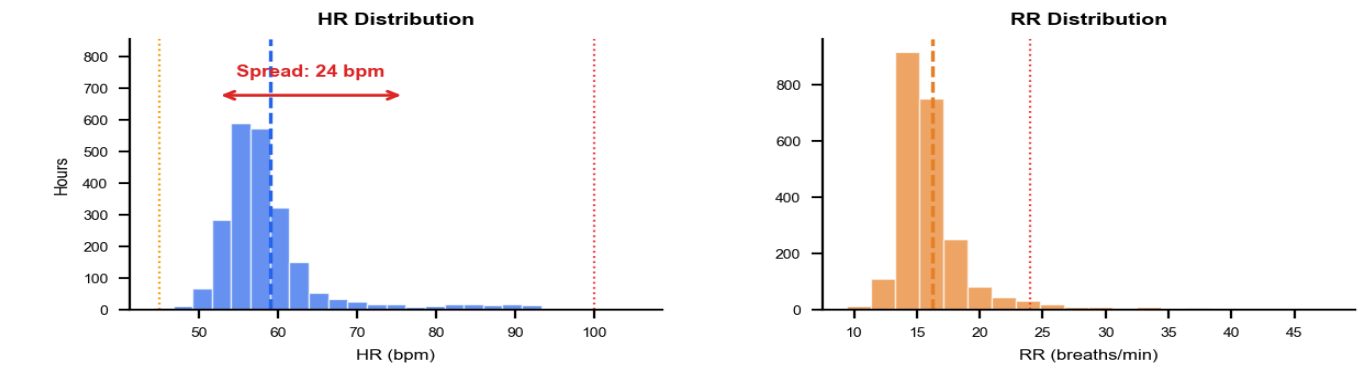
Clinical Pattern Observations:

- 1. **High variability:** HR spread 24 bpm (52 to 76).
- 2. **Clustered pattern:** Episodic events on 5 of 65 days (7%).
- 3. **Daytime pattern:** Episodes concentrated during waking hours.

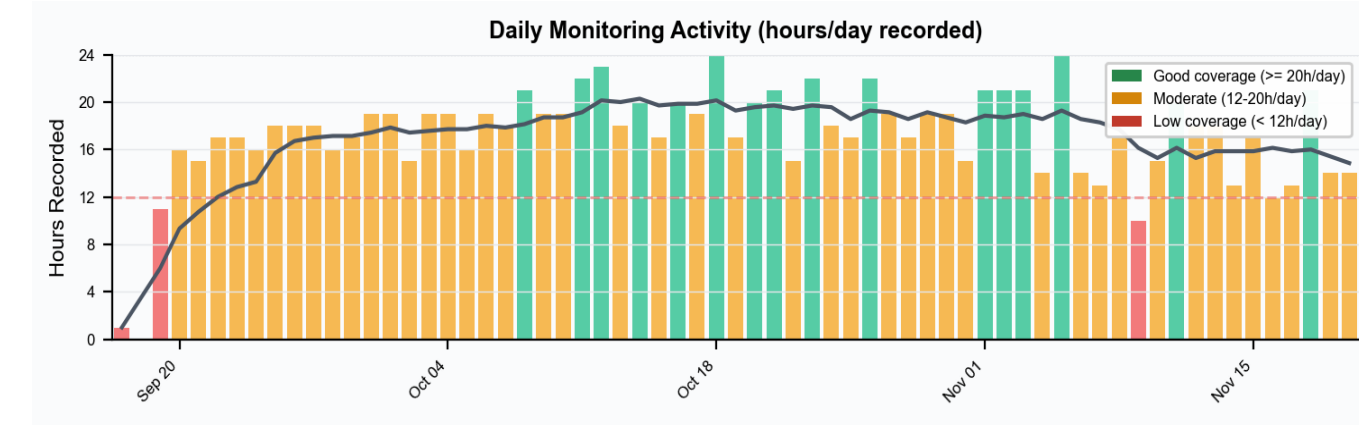


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.